



Prolonged Pregnancy

Objectives

By the end of this lecture students should be able to :

1. Define prolonged pregnancy.
2. Describe the possible causes for prolonged pregnancies.
3. Explain the risks of prolonged pregnancies.
4. Outline the management plan for prolonged pregnancy.



Post-term pregnancy :is a pregnancy progressing to 42 weeks (294)days of gestation or more from the first day of the last menstrual period

Many clinicians use the term prolonged pregnancy to describe pregnancies of 41 weeks of gestation and beyond

Post-date pregnancy:is a pregnancy lasting beyond the estimated date of delivery (EDD) at 40 weeks.

- The incidence is around 10%
- Use of u/s has lowered the incidence to 2-5%
- All pregnant ladies should have an ultrasound examination for gestational age determination prior to 16 weeks of gestation

Causes

- The most frequent cause is error in dating

Risk factors :

- Anencephaly
- Fetal adrenal hyperplasia
- Previous post term pregnancy
- Primiparity
- Genetic predisposition

Complications

Maternal :

- Labor dystocia
- Severe perineal injury
- Doubling in the rate of C/S
- Anxiety

Fetal risks :

- increased risk of intra-partum fetal death & neonatal death
- Post-maturity syndrome
- Meconium aspiration
- Macrosomia >4 kg

Post-maturity syndrome

Is a pathological syndrome seen in post-term fetuses experiencing placental insufficiency and IUGR

It affects 20% of cases and is associated with:

- Meconium stained amniotic fluid
- Oligohydramnios
- Fetal distress
- Loss of subcutaneous fat
- Dry, cracked skin

Prevention:

Membrane sweeping can be performed from 39 weeks onward to encourage spontaneous labor at term, As it increases the circulating prostaglandins.

Management at 40-41 weeks

Immediate delivery if there is :

- Maternal risk factors :pre-eclampsia, DM , RH- incompatibility
- Evidence of fetal compromise:

Decreased fetal movements, abnormal NST

Oligohydramnios by U/S ($<5\text{cm}$)

For healthy uncomplicated pregnancy offer follow up until 41 weeks of gestation and then IOL at 41 weeks.

If the mother declines IOL at 41 weeks:

- Daily fetal movement counts.
- Twice weekly NST and AFI measurement upto 42 weeks
- Once the woman reaches 42 weeks we have to deliver her even if NST and AFI are still normal

Management during labor and delivery:

- Consider amniotomy to diagnose meconium.
- Continuous CTG if meconium is present
- Be prepared for shoulder dystocia and neonatal resuscitation at delivery.

Conclusion

- effective management of post-term pregnancy requires balancing the risks of continuing pregnancy against those of intervention, with the primary goal of achieving a safe outcome for both the mother and baby.

References

1. NICE NG207 — “Inducing labour” (published 4 November 2021).
2. Obstetrics by ten teachers 21st edition